

*Dr. Ryan DeNome, D.C.*

# MONO DOESN'T ALWAYS LEAVE

*it just changes*

*its name*

*What Epstein Barr does after  
the sore throat goes away*

THE WELLNESS WAY · MASON

BEFORE YOU START

# Some things are not a terrain conversation.

Mono can cause complications that need same-day care. If any of this is happening, stop reading and get seen.

## **Pain in the upper left of your abdomen, under the ribs**

Mono can enlarge the spleen. This is the one people talk themselves out of.

## **Trouble breathing or swallowing**

Including swelling that makes it hard to get air.

## **Yellowing of your skin or eyes**

Get liver involvement looked at the same day.

## **Fainting, confusion, or a headache unlike any you have had**

Do not wait to see whether it passes.

## **You cannot keep fluids down**

Dehydration compounds fast when you are already depleted.

In the first weeks of mono, ask a physician before contact sports or heavy lifting. An enlarged spleen is why that advice exists.



# Before we start.

*You are not imagining this, and you are not the first person to sit where you are sitting.*

Most people meet Epstein Barr in high school. You feel wrecked for a few weeks, then everyone moves on.

What nobody tells you is that it does not clear the way a cold clears.

When the sore throat ends, the infection does not. The virus settles into your B cells, the cells your immune system uses to remember threats, and stays for life.

For almost everyone that is a non-event. Which is exactly why finding it in your blood explains so little on its own.

This guide is about the better question: what changed in your body that made keeping it quiet harder. Read it slowly. Bring it with you.

*Ryan*

DR. RYAN DENOME, D.C.



# The virus never left. So why you, and why now?

Nearly everyone reading this is carrying Epstein Barr right now. You did not catch it by doing anything wrong.

|                                                                                                |                                                                                                       |                                                                            |
|------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------|
| <b>90%+</b><br>of adults carry antibodies,<br>meaning they met it and their<br>body handled it | <i>For life</i><br>it stays in the body after a first<br>infection, held quiet rather<br>than removed | <i>Almost always</i><br>that is a non-event you never<br>think about again |
|------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------|

So finding it in your blood does not explain much. The useful question is not whether you have it, but what made keeping it quiet harder.

## **Holding a virus dormant is work.**

Your immune system is not ignoring this virus. It is actively holding a lid on it every day, and that job costs something. When enough other load stacks up, there is less left for the lid.

*Two people carry the same virus and only one ends up exhausted. Not because one is weaker, but because one is carrying more. So we do not ask what is wrong with your immune system. We ask what it has been busy with.*



# What stacks the load.

None of these are exotic. That is the point. They are the ordinary load a body carries for years without anyone adding it up.

- 1 **Chronic stress**  
The kind running for years, not the kind that ends on Friday.
- 2 **Sleep that never quite lands**  
Short nights, broken nights, or eight hours that leave you flat.
- 3 **Gut inflammation**  
Most of your immune system lives down there. A busy gut leaves the rest short-handed.
- 4 **Food reactions you cannot feel**  
A reaction need not be dramatic to be constant.
- 5 **Low vitamin D**  
Common in Ohio by February, and well studied in immune function.
- 6 **Blood sugar on a roller coaster**  
Metabolic stress is immune stress.
- 7 **Pregnancy and postpartum**  
Immune function shifts on purpose. Normal, and a real change in terrain.

*Think back to the months before your symptoms started. Not the week. The months.  
Most people can name it once they look.*



# Most of the work happens in your gut.

Roughly 70 to 80 percent of your immune tissue sits in and around your intestinal lining. That is anatomy, not a wellness slogan, and it is why a conversation that starts with a virus so often ends up nowhere near your throat.

1

## **A busy gut leaves the system short-handed**

An inflamed lining asks for immune attention every day. That is attention not available for holding a lid on a virus.

2

## **Food reactions are the quiet version**

Most people picture hives and an epi-pen. The common version is a food you eat three times a week that quietly costs you, and that you have never linked to how you feel.

3

## **This is testable, so we test it**

We measure food reactivity and gut function rather than guess. Elimination diets built on hunches remove foods you never needed to, and miss the ones costing

## **This is the part you can actually change**

You cannot un-have mono. You can change what your gut is dealing with while your immune system holds that lid, and unlike the virus, we can measure that before and after.



WHY NOBODY CAUGHT IT

# It almost never looks like mono.

The first time it was obvious. Fever, sore throat, a month on the couch.  
Reactivation does not announce itself that way, which is exactly why it gets missed.

- 1 **Fatigue coffee cannot touch**  
Not tired. Emptied. You sleep and wake up owing more.
- 2 **Thyroid antibodies climbing**  
The number moves before you feel anything change.
- 3 **Ferritin in the tank**  
Often alongside hair that sheds and a heart rate that runs high.
- 4 **Glands that swell and settle and swell**  
Neck, jaw, armpit. It comes and goes, so nobody chases it.
- 5 **Foods you used to be fine with**  
Flushing. Itching. A headache an hour later.
- 6 **Brain fog you started calling normal**  
Losing the word. Rereading the paragraph.

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*You are not lazy. You are not imagining it.*

*Your immune system is occupied.*



WHAT THIS ACTUALLY LOOKS LIKE

## Good day. Then you pay for it.

You feel better, so you do more. Two days later you are flattened. That pattern does not prove reactivation on its own, but it tells us something real about your capacity, and capacity is measurable.

WHAT SETS IT OFF

- Exercise you used to do without thinking
- A short night, or several
- A stretch of high stress
- Alcohol, or any illness on top

WHAT THE CRASH LOOKS LIKE

- Delayed a day or two, not that evening
- Out of proportion to the effort
- Days of recovery, not hours
- Same symptoms, same order

And underneath it, the part nobody asks about: work you are behind on, reading that takes three passes, drives you avoid, workouts you stopped explaining, friendships gone quiet, the person you were two years ago.

Write it down. Date, what you did, how you slept, what followed. Three weeks of that tells us more than any single lab value, and nobody else has been collecting it.





# Fine is not the same as thoroughly looked at.

A standard panel catches what is common and dangerous. It is not built to answer why you feel like this. With a mono history there are usually three gaps.

1

## **Nobody looked at the virus**

A standard panel does not include Epstein Barr antibodies. If it is never ordered, a reactivation cannot show up.

2

## **Nobody looked at the thyroid properly**

A TSH alone is not a thyroid panel. It is a signal from the brain, not a measure of what your thyroid makes or converts.

3

## **Nobody looked at what keeps the lid off**

Gut, food reactions, vitamin D, blood sugar, iron and ferritin.

### A PANEL CAN HELP WITH

- Whether you have met this virus
- Whether the pattern fits a recent infection
- A starting point read next to your history

### A PANEL CANNOT

- Give a yes or no on reactivation from one number
- Prove Epstein Barr causes how you feel
- Replace looking at thyroid, gut, iron, vitamin D

We would rather say that up front than sell you certainty a lab cannot deliver. The panel narrows the question. Your history answers it.



# Your results, in plain language.

These are the names people bring us, screenshotted from a portal with one value in red. Notice how often the honest answer is that it depends on the others.

|            |                                                                                                                                 |
|------------|---------------------------------------------------------------------------------------------------------------------------------|
| VCA IGM    | Rises early in a first infection, usually fades within weeks to months. Its absence rules out little.                           |
| VCA IGG    | Appears early and typically stays positive for life. Positive usually means you have met this virus, not that you are sick now. |
| EBNA-1 IGG | Develops weeks to months after a first infection and generally persists. Points to a past infection more than a new one.        |
| EA-D IGG   | The one that gets over-read, including by clinicians. Detectable in healthy people for years. Alone it confirms nothing.        |
| EBV PCR    | Detects viral genetic material rather than antibodies. Useful in specific situations, especially if you are immunocompromised.  |

No single line here answers your question. The pattern across all of them, read next to when you got sick and how you function today, is the thing worth paying for.



THE PATTERN WE KEEP SEEING

# Why the thyroid keeps showing up.



Of all the places an Epstein Barr conversation ends up, the thyroid is the most common, and it is not a coincidence. The thyroid is warm and richly supplied with blood, which makes it hospitable. When we suspect a viral load we look there as a matter of course.

## **A TSH on its own is not a thyroid panel**

TSH is a message from the pituitary telling the thyroid what to do. It is not a measure of what your thyroid produces, or whether you convert it to the active form. A complete picture includes free and total T3 and T4, reverse T3, and thyroid antibodies.

*This is why a question that starts with a virus so often turns into a thyroid finding, and why running one without the other leaves you half an answer.*



WHAT IS AHEAD, AND WHAT IS YOURS

# You cannot un-have mono.

Most people who get mono do return to their baseline, slower than they wanted. A meaningful minority run months of symptoms. That is documented, and it is not evidence that something in you is permanently broken. Persistent is not the same as permanent.

If you have spent nights reading about ME/CFS, we understand how you got there. We would rather you bring that fear to someone who will actually look.

## NOT UP TO YOU

- That you were infected
- That the virus stays latent for life
- Your genetics
- That it happened when it did

## UP TO YOU, AND MEASURABLE

- What your gut deals with daily
- Which foods you react to
- Vitamin D, iron, ferritin
- Whether blood sugar is steady
- Sleep, protected like a prescription

### On supplements and detoxes

When nobody gives you a plan, you build one, and the supplement aisle offers the agency you have been denied. But money spent on a protocol built for someone else's body is how people lose a year. Find out what is happening first.



WHAT WE DO

# How this goes.

1

## **We take the history nobody took**

When you got sick, how long it took to come back, and what was happening the year things shifted. Bring recent labs and we read them rather than repeat them.

2

## **We run the markers**

Epstein Barr antibodies, plus what the rest of your system is doing: thyroid with antibodies, iron and ferritin, vitamin D, blood sugar and insulin, food reactivity, gut function. You see the reasoning and the cost first.

3

## **We go through results with you**

Properly, in plain language. You leave understanding what we found, not holding a printout.

4

## **We go at the pace your body will take**

An immune system already working overtime tends to backfire when pushed hard, so we do not.

## **Where to start before any of that**

Protect sleep like a prescription. Put protein at the front of every meal. Get vitamin D checked by late winter. None of this is a protocol and none of it replaces knowing what is going on. It just stops adding to the load.



BRING THIS WITH YOU

# Three things worth writing down.

Ten minutes on this before any appointment, ours or anyone else's, is the difference between fifteen minutes of recall and a conversation that goes somewhere.

1

## **Your timeline**

When you had mono. When you last felt like yourself. What was happening in the year before things shifted.

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2

## **Your function, not just symptoms**

What you have stopped doing. What takes longer. What you need to recover from.

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3

## **Everything you are taking**

Medications, supplements, protocols, with doses.

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Bring your prior labs. If it was run in the last year, we would rather read it than repeat it.



ASK THESE OF US, OR OF ANYONE

# Ask harder questions.

If a practitioner cannot answer these plainly, that tells you something useful.

- “ *Does this fit a recent infection, a past one, or something else?*
- “ *If we run EBV testing, what will it answer, and what will it still not tell us?*
- “ *If the virus does not account for how I feel, what else are we checking?*
- “ *What are we testing beyond the virus: gut, food reactivity, thyroid, ferritin,*
- “ *vitamin D?*
- “ *What does this cost, and what will you do differently depending on the result?*

*That last one matters most. A test that will not change what happens next is a test worth skipping.*

Your next step is a discovery consultation to see if we can help and then we schedule a new patient appointment! We go through your history, your symptoms, and whatever labs you have, and decide together what is worth running. No guessing.

thewellnesswaymason.com · (283) 223-8376

